

PPI Market Commercial Analytics

Market Growth, Competitive Dynamics, Recorded Cost and ICB Market Development in England

Executive Summary

This project develops a Power BI commercial analytics dashboard for the English Proton Pump Inhibitor (PPI) market using publicly available NHS prescribing and GP population data.

The analysis focuses on five major PPI molecules:

- Omeprazole
- Lansoprazole
- Esomeprazole
- Pantoprazole
- Rabeprazole

The project evaluates market performance across five analytical layers:

1. category growth;
2. molecule-level competitive performance;
3. prescription-volume versus recorded NIC dynamics;
4. population-adjusted ICB prescribing intensity; and
5. ICB market-development segmentation.

The primary comparison is **January–July 2026 versus January–July 2025**, while historical trends cover **January 2025 to July 2026**.

For Jan–Jul 2026, the defined PPI market recorded approximately **45.5 million prescription items** and grew by approximately **0.23% year-on-year**. This indicates a broadly stable market at total-category level.

However, molecule-level analysis identified significant internal competitive movement. Lansoprazole represented approximately **48.86% of prescription-item volume**, grew by around **2.64% YoY**, and gained approximately **1.14 percentage points of share**. Omeprazole declined over the same period.

Lansoprazole added approximately **571,000 prescription items**, while Omeprazole declined by approximately **517,000 items**. These opposing movements resulted in only modest overall category growth of around **106,000 prescription items**.

Recorded Net Ingredient Cost also moved differently from volume. Total NIC declined by approximately **2.55% YoY**, despite prescription items increasing slightly. Lansoprazole accounted for approximately **48.86% of prescribing volume but only 27.49% of recorded NIC**, producing a value-volume gap of approximately **-21.36 percentage points**.

A significant data-quality issue was identified during ICB-level analysis. Initial year-on-year results showed several artificial ICB declines of approximately **-56% to -58%**. Investigation identified changes in ICB structure and practice-to-ICB assignment during 2026 as the source of the inconsistency.

To resolve this, the geographic model was rebuilt using **July 2026 NHS practice-to-ICB reference data**, harmonising prescribing and population analysis to a consistent ICB structure. This improved the comparability of ICB-level YoY growth and population-adjusted Market Development Index measures.

The final dashboard contains five pages:

1. Market Overview
2. Competitive Performance
3. Volume vs Value
4. Geographic Market Development
5. ICB Market Segmentation

The project demonstrates pharmaceutical commercial analytics, Power Query, DAX, dimensional modelling, data-quality investigation, geographic harmonisation, population-adjusted benchmarking, Figma-supported dashboard design and commercial interpretation.

1. Introduction

Commercial pharmaceutical analysis often requires more than monitoring total market size.

A market can remain stable overall while undergoing significant molecule-level substitution. Prescription volume may also behave differently from recorded ingredient cost, while geographic prescribing intensity can vary substantially after adjusting for population.

This project applies these analytical principles to the English Proton Pump Inhibitor market.

The project was intentionally designed as a multi-layer commercial analysis rather than a simple market tracker.

The analytical framework is:

Market → Molecule → Growth → Share → Recorded Cost → Geography → Market Development

The project therefore combines:

- national category analysis;
- competitive molecule analysis;
- recorded cost analysis;
- geographic prescribing analysis; and
- population-adjusted benchmarking.

The objective is to produce a commercial analytics tool capable of explaining both:

what is happening in the PPI market

and:

where meaningful differences exist across molecules and ICBs.

2. Business Context and Analytical Question

The project assumes the role of a commercial analyst reviewing the English PPI market.

The primary business question is:

How is the PPI prescribing market evolving across England, which molecules are gaining or losing competitive share, and how does prescribing intensity vary across ICBs?

Supporting questions include:

1. How large is the PPI market?
2. Is the category growing or declining?
3. Which molecule leads prescription-item volume?
4. Which molecules are gaining or losing share?
5. Which molecules contribute most to net category growth?
6. How does recorded NIC differ from prescription-volume dynamics?
7. Which molecules have disproportionately high or low NIC share relative to volume share?
8. How does prescribing intensity vary after adjusting for registered population?
9. Which ICBs sit above or below the England prescribing-intensity benchmark?
10. Which ICBs combine relatively strong or weak growth with high or low market development?

3. Data Sources

The project combines three publicly available NHS data sources.

3.1 NHS Business Services Authority English Prescribing Dataset

The English Prescribing Dataset provides monthly prescribing activity across England.

Relevant fields include:

- prescribing month;
- practice code;
- chemical substance code;
- molecule name;
- presentation code;
- presentation description;
- prescription items;
- total quantity;
- Net Ingredient Cost;
- Actual Cost; and
- SNOMED code.

The dataset forms the primary commercial fact table.

3.2 Patients Registered at a GP Practice — July 2026

The July 2026 GP population dataset provides the registered population denominator used for geographic analysis.

The relevant fields are:

PracticeCode;

RegisteredPopulation.

This population snapshot is used to calculate prescribing intensity per 1,000 registered population.

3.3 July 2026 Practice-to-ICB Mapping

The practice mapping dataset provides a consistent July 2026 assignment between:

- GP practice;
- ICB code;
- ICB name.

This source became particularly important during the data-quality investigation because historical prescribing geography was not directly comparable across all periods.

4. Analytical Scope

4.1 Market Definition

The defined PPI market consists of five molecules:

- Omeprazole
- Lansoprazole
- Esomeprazole
- Pantoprazole
- Rabeprazole

The primary analysis is conducted at molecule level.

Therefore:

Market share refers to prescription-item share within the defined five-molecule PPI category.

4.2 Time Period

The final analytical period covers:

January 2025 – July 2026

This provides 19 monthly periods.

Headline comparison:

January–July 2026 vs January–July 2025

Historical trend period:

January 2025 – July 2026

This period was selected to retain year-on-year comparability while keeping the Power BI model sufficiently responsive.

5. Data Preparation

The raw monthly prescribing files were prepared in Power Query.

The main workflow included:

1. importing monthly prescribing datasets;
2. keeping required analytical fields;
3. harmonising source schemas;

4. standardising column names;
5. correcting data types;
6. converting monthly values into date fields;
7. filtering to the five PPI molecules;
8. appending monthly datasets;
9. building dimension tables;
10. importing population data;
11. importing practice-to-ICB reference data;
12. harmonising geography;
13. validating model relationships;
14. testing national and ICB-level measures.

5.1 Source Field Standardisation

The following fields were standardised:

- YEAR_MONTH → YearMonth
- ICB_CODE → ICBCode
- ICB_NAME → ICBName
- PRACTICE_CODE → PracticeCode
- BNF_CHEMICAL_SUBSTANCE → ChemicalCode
- CHEMICAL_SUBSTANCE_BNF_DESCR → Molecule
- BNF_CODE → PresentationCode
- BNF_DESCRIPTION → PresentationName
- ITEMS → Items
- TOTAL_QUANTITY → TotalQuantity
- ACTUAL_COST → ActualCost
- SNOMED_CODE → SNOMEDCode

5.2 Date Transformation

Source month values stored in YYYYMM format were converted into proper date values.

For example:

202501 → 01 January 2025

This supported:

- chronological sorting;
- monthly trends;

- previous-year measures;
- year-on-year growth analysis.

5.3 Financial Data-Type Handling

NIC and Actual Cost required careful numeric parsing.

Both were explicitly converted to decimal numeric types before downstream modelling.

Fixed divisors were not applied.

This reduced the risk of artificially scaling financial fields and preserved the original decimal structure.

5.4 Presentation Attributes

A presentation dimension was created to support future product-level analysis.

The following attributes were derived from presentation names:

- Formulation
- Strength
- PresentationGroup

Examples include:

- Capsule 20 mg
- Tablet 40 mg
- Oral Solution
- Granules

Although the final dashboard remains molecule-focused, this structure provides flexibility for future extension.

6. Data Model

The Power BI model follows a dimensional structure centred on prescribing activity.

Core tables include:

- Fact_Prescribing
- Dim_Calendar
- Dim_Chemical
- Dim_Presentation
- Dim_Practice

- Dim_ICB
- Dim_PracticePopulation

The core model logic is:

Calendar / Chemical / Presentation / Practice → Fact_Prescribing

while population analysis is connected through:

Dim_ICB → Dim_PracticePopulation

6.1 Fact_Prescribing

The fact table contains:

- YearMonth
- PracticeCode
- ChemicalCode
- Molecule
- PresentationCode
- PresentationName
- Items
- TotalQuantity
- NIC
- ActualCost
- SNOMEDCode

Historical ICB fields were retained for quality assurance but hidden from report view once the harmonised geography model was established.

6.2 Dim_Calendar

Contains:

- YearMonth
- Year
- MonthNumber
- MonthName
- Quarter
- MonthYear
- YearMonthSort

6.3 Dim_Chemical

Contains:

- ChemicalCode
- Molecule
- TherapyClass
- PPIFlag

6.4 Dim_Presentation

Contains:

- PresentationCode
- PresentationName
- ChemicalCode
- Molecule
- SNOMEDCode
- Formulation
- Strength
- PresentationGroup

6.5 Dim_Practice

Contains:

- PracticeCode
- ICBCode

Each practice is assigned to the harmonised July 2026 ICB structure.

6.6 Dim_ICB

Contains:

- ICBCode
- ICBName

This dimension represents the geographic structure used consistently across the final analysis.

6.7 Dim_PracticePopulation

Contains:

- PracticeCode
- ICBCode
- RegisteredPopulation

This provides the population denominator for ICB-level benchmarking.

7. Data Quality Investigation and ICB Geography Harmonisation

One of the most important stages of the project was identifying and resolving an inconsistency in ICB-level year-on-year analysis.

7.1 Initial QA Finding

During geographic QA, several ICBs showed apparent year-on-year declines of approximately:

-56% to -58%

The magnitude and repetition of these declines across multiple areas suggested that the issue was unlikely to represent genuine prescribing behaviour.

This triggered a deeper investigation.

7.2 Root Cause

The analytical period spans organisational changes to ICB geography during 2026.

Historical prescribing data reflected geography associated with the structure active when the records were produced.

The July 2026 population denominator and current ICB structure reflected later practice-to-ICB assignments.

This created an apples-to-oranges comparison.

In effect:

2025 prescribing and 2026 prescribing were not always being grouped through the same ICB structure.

As a result, some historical ICBs appeared to lose large amounts of prescribing activity even when the change was primarily structural.

7.3 Resolution

The geographic model was rebuilt using the July 2026 NHS practice-to-ICB mapping.

The harmonised logic became:

PracticeCode → July 2026 ICBCode → July 2026 ICBName

This same geography was applied to:

- Jan–Jul 2025 prescribing;

- Jan–Jul 2026 prescribing; and
- July 2026 registered population.

This provided a consistent basis for year-on-year ICB comparison.

7.4 Model Redesign

The direct historical relationship between ICB geography and the prescribing fact table was no longer used for final geographic analysis.

Instead:

Dim_ICB → Dim_Practice → Fact_Prescribing

was used for prescribing.

Population used:

Dim_ICB → Dim_PracticePopulation

This eliminated the inconsistent historical geography path.

It also avoided ambiguous relationships within Power BI.

7.5 Post-Fix Validation

After rebuilding the geographic model:

- the artificial -56% to -58% declines disappeared;
- ICB growth distributions became materially more credible;
- Items per 1,000 remained internally consistent;
- England-level totals continued to reconcile;
- Market Development Index measures behaved as expected.

This stage was critical because it demonstrated that:

technically correct calculations can still produce analytically misleading results when the underlying geography is inconsistent.

8. Commercial KPI Framework

The project uses multiple complementary measures.

8.1 Total Items

Prescription Items represent prescribing activity and are used as the primary volume measure.

They do not represent unique patients.

8.2 Year-on-Year Growth

YoY Growth measures:

$$\text{(Current Items – Prior-Year Items) / Prior-Year Items}$$

This describes the rate of change.

8.3 Absolute Growth

Absolute Growth measures:

$$\text{Current Items – Prior-Year Items}$$

This describes the actual amount of prescribing volume added or lost.

8.4 Volume Share

Volume Share measures:

$$\text{Molecule Items / Total PPI Market Items}$$

This represents molecule-level prescription-item share within the defined PPI market.

8.5 Share Change

Share Change measures:

$$\text{Current Volume Share – Prior-Year Volume Share}$$

The result is expressed in percentage points.

8.6 Growth Gap

Growth Gap measures:

$$\text{Molecule YoY Growth – PPI Market YoY Growth}$$

A positive value indicates that the molecule is growing faster than the category.

8.7 Contribution to Net Category Growth

Contribution is calculated as:

$$\text{Molecule Absolute Growth / PPI Absolute Growth}$$

The measure can exceed 100% or become negative when positive and negative molecule movements offset each other.

8.8 NIC

NIC is used as the primary recorded ingredient-cost measure.

It should not be interpreted as manufacturer revenue or profitability.

8.9 NIC per Item

NIC per Item measures:

NIC / Prescription Items

This provides a recorded cost-intensity indicator.

It is not an average selling price.

8.10 NIC Share

NIC Share measures:

Molecule NIC / Total PPI NIC

8.11 Value-Volume Gap

Value-Volume Gap is defined as:

NIC Share – Volume Share

A positive value indicates that NIC share exceeds volume share.

A negative value indicates the opposite.

9. Population-Adjusted Measures

9.1 Registered Population

The project uses July 2026 registered GP population.

This is treated as a static denominator for current geographic benchmarking.

9.2 PPI Items per 1,000

The measure is:

PPI Items / Registered Population × 1,000

This adjusts prescribing volume for population size.

9.3 England PPI Items per 1,000

The England benchmark is calculated by removing the ICB filter.

For Jan–Jul 2026, the national value was approximately:

717.6 PPI items per 1,000 registered population

9.4 Market Development Index

Population MDI is:

ICB Items per 1,000 / England Items per 1,000 × 100

Interpretation:

- MDI = 100 → equal to England
- MDI > 100 → above England
- MDI < 100 → below England

MDI is a relative prescribing-intensity benchmark.

10. Dashboard Design

The dashboard was developed using a combined Figma and Power BI workflow.

Figma was used for:

- header structure;
- page navigation;
- background layout;
- visual consistency.

Power BI was used for:

- KPI cards;
- DAX measures;
- time intelligence;
- molecule selection;
- geographic benchmarking;
- ICB segmentation;
- interactive navigation.

The final navigation contains:

- Overview
- Competition
- Value
- Geography
- Segmentation

11. Results

11.1 Page 1 — Market Overview

The first page answers:

What is happening to the PPI market overall?

Headline metrics for Jan–Jul 2026 include:

- Total PPI Items: approximately 45.5M
- PPI YoY Growth: +0.23%
- Total NIC: approximately £91.6M
- NIC YoY Growth: -2.55%
- Leading Molecule: Lansoprazole
- Leading Molecule Share: approximately 48.86%

The page also contains:

- monthly market trend;
- current molecule mix;
- market share trend.

Interpretation

The category remained broadly stable in prescribing volume.

However, competitive composition continued to change.

Lansoprazole represented almost half of total PPI prescription items and strengthened its position relative to the previous year.

Recorded NIC declined despite slightly higher prescription volume, indicating that cost dynamics differed from volume dynamics.

11.2 Page 2 — Competitive Performance

The second page evaluates individual molecule performance relative to the market.

For Lansoprazole:

- Prescription Items: approximately 22.2M
- YoY Growth: +2.64%
- PPI Market Growth: +0.23%
- Growth Gap: approximately +2.40 pp

- Volume Share: 48.86%
- Share Change: approximately +1.14 pp

Absolute Growth

Lansoprazole added approximately:

+571K items

while Omeprazole declined by approximately:

-517K items

The entire market increased by only around:

+106K items

Interpretation

The overall PPI market looked stable, but large internal molecule-level movements were occurring.

Lansoprazole growth substantially offset Omeprazole decline.

This demonstrates why category growth alone is insufficient for explaining competitive dynamics.

11.3 Page 3 — Volume vs Value

The third page evaluates recorded cost alongside prescribing volume.

For Lansoprazole:

- Volume Share: 48.86%
- NIC Share: 27.49%
- Value-Volume Gap: approximately -21.36 pp
- NIC per Item: approximately £1.13

Omeprazole represented a smaller share of prescription volume than Lansoprazole but a substantially larger share of recorded NIC.

Esomeprazole also showed a relatively high NIC share compared with volume share.

Interpretation

The largest molecule by prescription volume is not necessarily the largest molecule by recorded ingredient cost.

The differences are associated with variation in recorded NIC per item and prescribing mix.

These measures should not be interpreted as revenue or profitability.

11.4 Page 4 — Geographic Market Development

The fourth page evaluates population-adjusted prescribing intensity across ICBs.

The key benchmark is:

England PPI Items per 1,000 = approximately 717.6

ICBs are then indexed relative to England using Population MDI.

The page includes:

- highest-MDI ICBs;
- lowest-MDI ICBs;
- ICB YoY growth;
- England growth benchmark.

Interpretation

ICBs vary substantially in prescribing intensity even after adjusting for registered population.

However, higher prescribing intensity does not automatically represent better market performance or stronger clinical opportunity.

Differences may reflect:

- local population structure;
- underlying GI disease burden;
- prescribing pathways;
- formularies;
- treatment duration;
- deprescribing initiatives.

The metric is therefore used as:

a relative market-development benchmark rather than a direct opportunity score.

11.5 Page 5 — ICB Market Segmentation

The final page combines market development and prescribing growth.

The segmentation uses:

X-axis

Population MDI

Y-axis

ICB YoY Growth

Bubble size

Total PPI Items

Reference lines:

- Population MDI = 100
- England YoY Growth = approximately +0.23%

The quadrants are:

Emerging Market

MDI below 100 and growth above England.

Expansion Market

MDI above 100 and growth above England.

Investigate

MDI below 100 and growth below England.

Mature Market

MDI above 100 and growth below England.

Interpretation

The segmentation provides a comparative framework rather than a definitive commercial-opportunity model.

An ICB in the Emerging Market quadrant may warrant further investigation, but lower prescribing intensity could reflect structural or clinical factors rather than unmet demand.

Similarly, high-MDI ICBs may simply reflect a population with greater prescribing need.

12. Overall Commercial Interpretation

The combined analysis produces a multi-layer commercial story.

First, the PPI market is mature and broadly stable.

Total prescription items increased by only around **0.23%**.

Second, this stability masks substantial competitive substitution.

Lansoprazole strengthened its position while Omeprazole declined.

Third, recorded NIC moved differently from prescribing volume.

NIC declined by approximately **2.55%**, and molecule-level NIC shares differed substantially from volume shares.

Fourth, prescribing intensity varies geographically after adjusting for population.

Finally, combining Market Development Index and growth provides a structured way to compare ICBs without automatically interpreting prescribing intensity as unmet clinical need.

The overall analytical story is:

Stable market → internal molecule substitution → volume-cost divergence → geographic prescribing variation → population-adjusted segmentation

13. Critical Evaluation

13.1 Prescription Items as a Volume Measure

Prescription items are suitable for measuring prescribing activity but do not represent patient counts.

Different formulations and treatment patterns may also affect item volumes.

13.2 NIC Interpretation

NIC provides useful recorded cost information but does not reflect:

- net manufacturer revenue;
- realised sales price;
- margin;
- rebates;
- profitability.

13.3 Geographic Benchmarking

Population adjustment improves comparability between ICBs, but registered population alone does not fully explain clinical demand.

Additional variables such as:

- age structure;
- disease prevalence;
- local prescribing pathways; and
- formulary policy

could improve future analysis.

13.4 Harmonised Geography

Restating historical prescribing to the July 2026 ICB structure improves comparability but differs from reporting performance using the exact historical organisational structure in each period.

This is a deliberate analytical trade-off.

14. Limitations

Prescription Items Are Not Patients

Prescription-item counts measure prescribing activity rather than unique patients.

Market Share Is Item-Based

Market share does not represent patient share or therapeutic-equivalent share.

NIC Is Not Revenue

Recorded NIC should not be interpreted as sales or profitability.

NIC per Item Is Not Average Selling Price

The measure is used only as a recorded cost-intensity indicator.

Static Population Denominator

The project uses July 2026 registered population rather than monthly population values.

Population MDI Is Not Treatment Penetration

MDI provides a relative prescribing-intensity benchmark.

It should not be interpreted as a direct estimate of unmet need or treatment penetration.

Geography Is Harmonised

Historical prescribing is restated using July 2026 practice-to-ICB geography.

England Scope

The analysis covers English NHS prescribing rather than total UK pharmaceutical consumption.

No Causal Attribution

The project identifies patterns but does not establish causal explanations.

15. Skills Demonstrated

The project demonstrates practical experience in:

- pharmaceutical commercial analytics;
- category-performance analysis;
- molecule-level competitive analysis;
- market-share analysis;
- growth-gap analysis;
- growth contribution analysis;
- recorded-cost analysis;
- volume-versus-value analysis;
- geographic analytics;
- population-adjusted benchmarking;
- Market Development Index modelling;
- ICB segmentation;
- Power Query;
- DAX;
- time intelligence;
- dimensional modelling;
- data-quality assurance;
- geography harmonisation;
- root-cause investigation;
- Figma-supported dashboard design;
- analytical storytelling.

A particularly important learning outcome was:

A technically correct calculation can still be analytically misleading if the underlying geography is inconsistent.

16. Conclusion

This project developed a Power BI commercial analytics dashboard for the English PPI market.

The analysis identified a category that was broadly stable at total-market level but undergoing meaningful molecule-level change.

For Jan–Jul 2026, approximately **45.5 million PPI prescription items** were recorded, representing approximately **+0.23% YoY growth**.

Lansoprazole strengthened its competitive position, reaching approximately **48.86%** volume share, growing by **2.64% YoY**, and gaining approximately **1.14 percentage points of share**.

The relatively small market increase concealed much larger molecule-level movements, with Lansoprazole adding approximately **571,000 prescription items** while Omeprazole declined by approximately **517,000 items**.

The project also identified substantial differences between prescription volume and recorded NIC.

Lansoprazole accounted for nearly half of prescription volume but only around **27.49% of recorded NIC**, while Omeprazole represented a disproportionately larger share of recorded cost.

At geographic level, population-adjusted analysis revealed meaningful variation in prescribing intensity across ICBs.

However, the most important methodological development occurred during data quality assurance.

Initial ICB-level results showed artificial declines of approximately -56% to -58%. Rather than treating these results as genuine commercial performance, the geographic structure was investigated and rebuilt using July 2026 practice-to-ICB mapping.

This process reinforced the importance of validating analytical comparability before interpreting outputs.

Overall, the project demonstrates how publicly available NHS data can be transformed into a structured commercial analytics framework combining:

- market performance;
- molecule competition;
- recorded-cost analysis;
- population-adjusted geography;
- and ICB market segmentation.

Appendix A — Selected DAX Measures

Total Items

Total Items =

SUM(Fact_Prescribing[Items])

NIC per Item

NIC per Item =

DIVIDE(

[NIC],

[Total Items]

)

PPI Market Items

PPI Market Items =

CALCULATE(

[Total Items],

REMOVEFILTERS(Dim_Chemical)

)

Volume Share

Volume Share =

DIVIDE(

[Total Items],

[PPI Market Items]

)

Growth Gap

Growth Gap (pp) =

VAR MoleculeGrowth =

[Items YoY Growth]

VAR MarketGrowth =

[PPI Market YoY Growth]

RETURN

IF(

NOT ISBLANK(MoleculeGrowth)

&& NOT ISBLANK(MarketGrowth),

(MoleculeGrowth - MarketGrowth) * 100

)

Contribution to Net Category Growth

Contribution to Net Category Growth =

VAR MoleculeGrowth =

[Absolute Items Growth]

VAR MarketGrowth =

[PPI Absolute Growth]

RETURN

IF(

NOT ISBLANK(MoleculeGrowth)

&& NOT ISBLANK(MarketGrowth),

DIVIDE(

MoleculeGrowth,

MarketGrowth

)

)

NIC Share

NIC Share =

DIVIDE(

[NIC],

[PPI Market NIC]

)

Value-Volume Gap

Value-Volume Gap (pp) =

([NIC Share] - [Volume Share]) * 100

PPI Items per 1,000

PPI Items per 1,000 =

DIVIDE(

[Total Items],

[Registered Population]

) * 1000

Population MDI

Population MDI =

DIVIDE(

[PPI Items per 1,000],

[England PPI Items per 1,000]

) * 100

Appendix B — Figures

Figure 1. Power BI Data Model

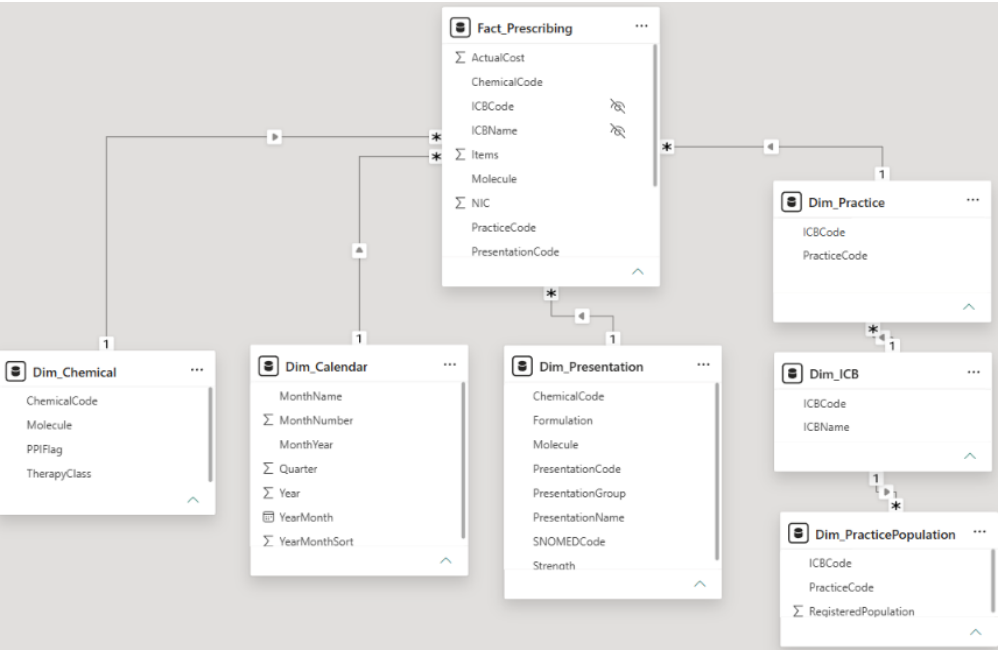


Figure 2. PPI Market Overview

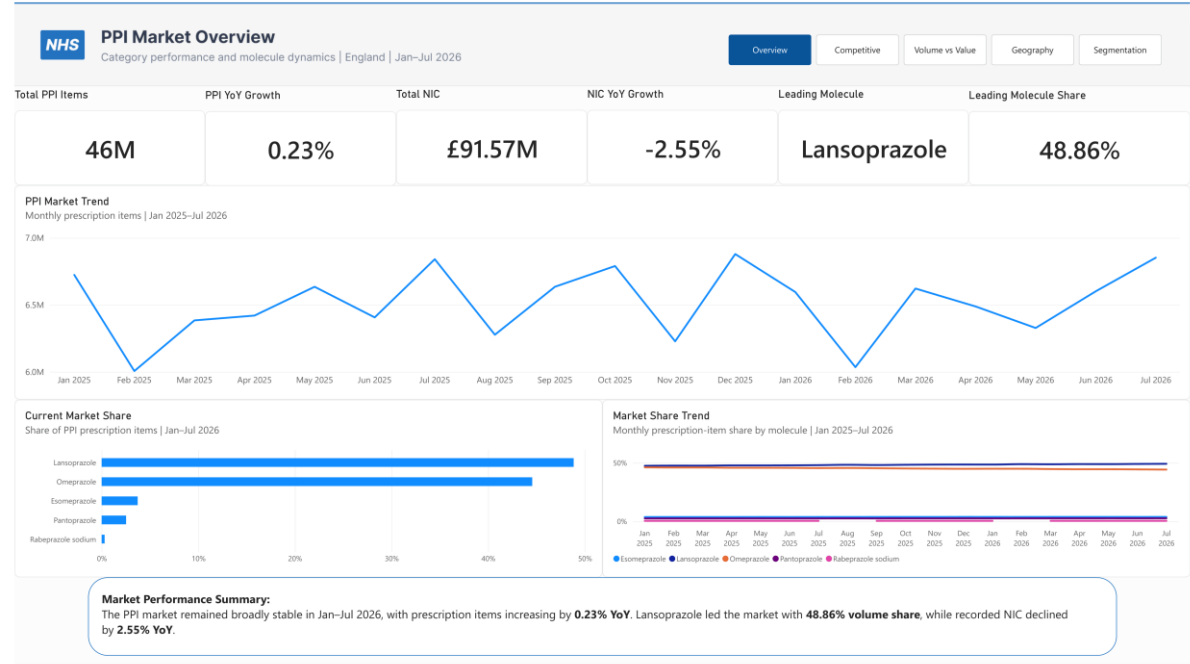


Figure 3. Competitive Performance

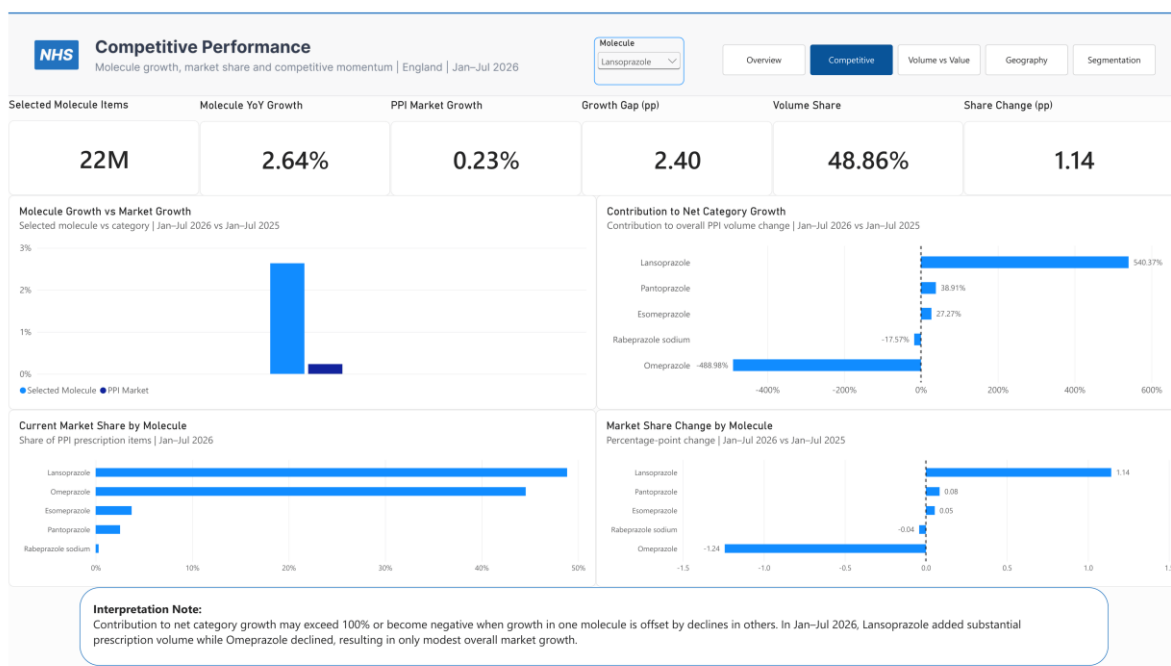


Figure 4. Volume vs Value

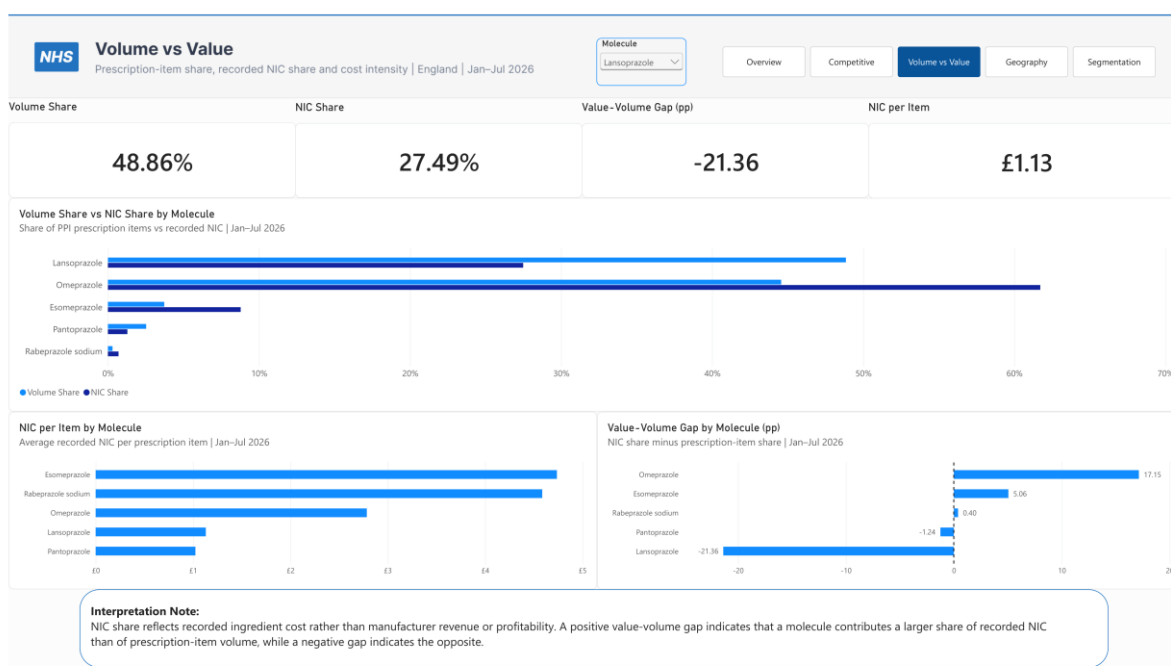


Figure 5. Geographic Market Development

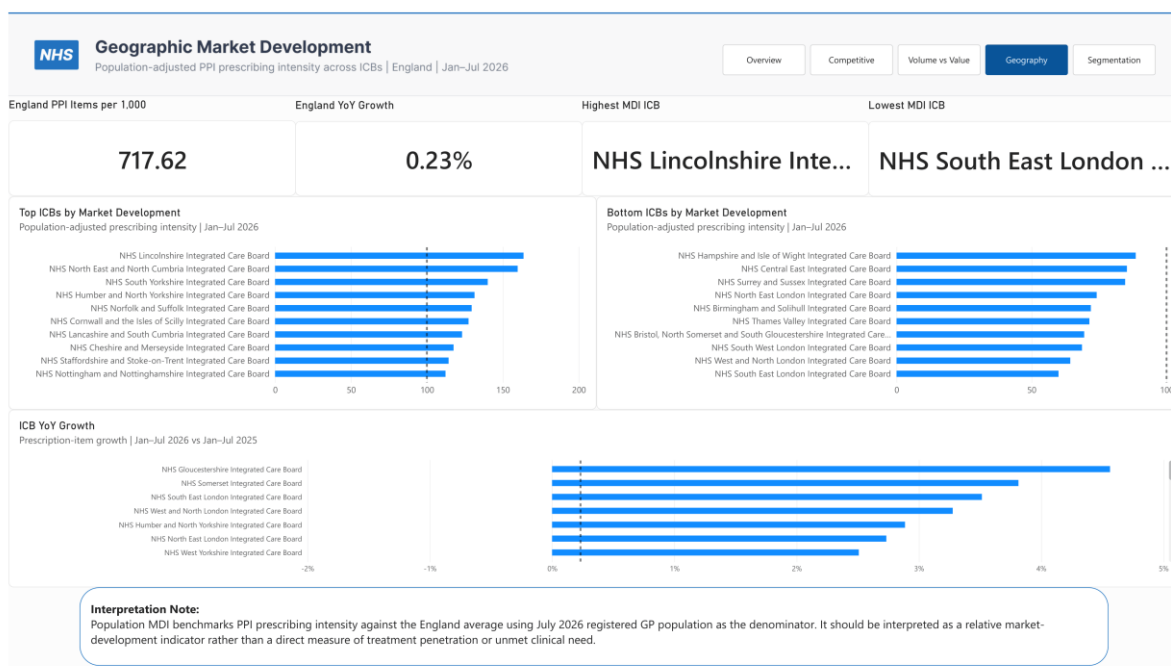
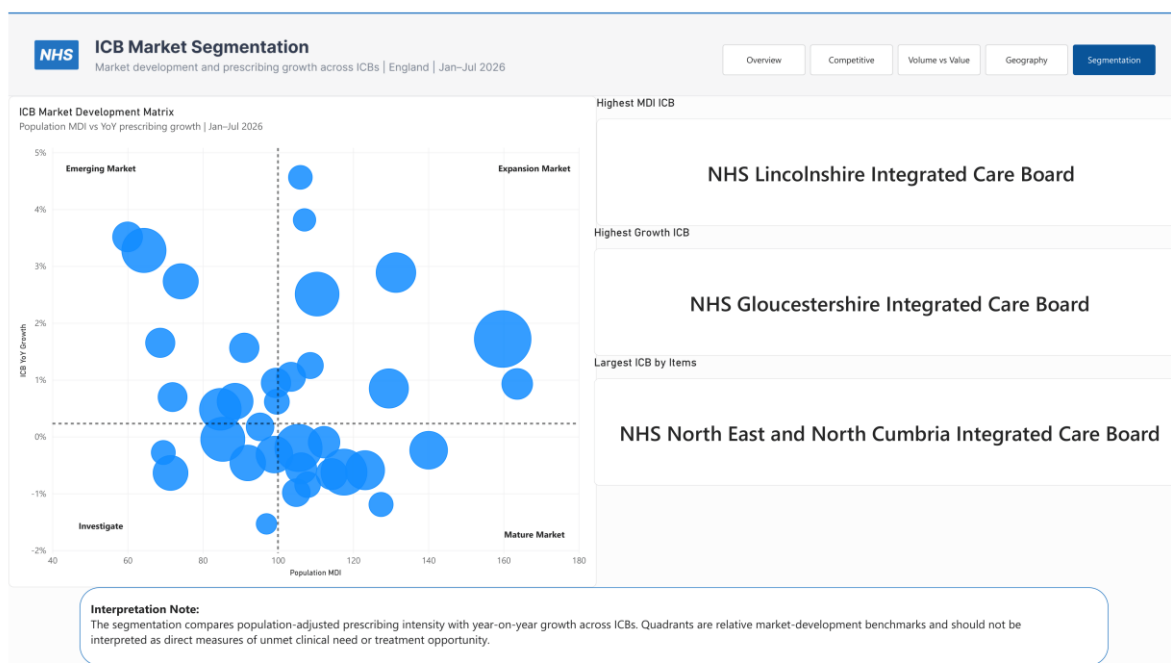


Figure 6. ICB Market Segmentation



Data Sources

NHS Business Services Authority — **English Prescribing Dataset**

NHS England / NHS Digital — **Patients Registered at a GP Practice, July 2026**

NHS practice-to-ICB mapping — **July 2026**

Disclaimer

This project was developed independently for educational and portfolio purposes using publicly available NHS data.

It is not affiliated with, endorsed by, or produced on behalf of the NHS, any pharmaceutical manufacturer, or any other commercial organisation.

The analysis is intended to demonstrate pharmaceutical commercial analytics, Power BI, data modelling, DAX, data-quality assurance and dashboard-design capabilities.

It should not be interpreted as clinical, financial, investment or commercial advice.